BASELINE → TREATMENT → FOLLOW-UP

# Safety tests, symptoms and response review

The team books and reviews each test. Mandatory medicine-safety checks are distinct from individually timed MRI and tumour-tissue tests. There is no universal blood tumour marker for these treatment cycles.

| Check | Before starting | During treatment | Between visits / recovery | Next-cycle / response review | After a pause or stop |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** | FBC, renal/liver profile, glucose, HBsAg and anti-HBc | FBC weekly during three-week RT | Continue FBC weekly for four weeks after RT; renal/LFT at 1 and 4 weeks after RT | Day 1 FBC/renal/LFT; cycle 1 can use four-week post-RT review. EMA Day 22 FBC | LFT after cycles; follow delayed toxicity after the last dose |
| **Neurology / medicines / function** | Record neurological symptoms, daily function and current medicines | Review seizures, headache, cognition, mood, walking and any steroid plan | Report changes early; do not wait for the next MRI | Review treatment tolerance, medication support and patient goals | Agree follow-up and rehabilitation or symptom support |
| **MRI / diagnosis** | Confirm integrated diagnosis, molecular results and baseline imaging | MRI timing according to the neuro-oncology plan | Keep the booking and result-review contact in your passport | Interpret scan with symptoms, treatment effects and clinical review | Agree the next imaging and clinical review |

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00461 v3 · current Irish regimen · NCCN Guidelines for Patients: Brain Cancer Glioma, 2026 · patient education reference · EMA Temodal product information and patient leaflet

THE WHOLE TREATMENT JOURNEY

# Temozolomide with short-course radiotherapy, then adjuvant treatment: treatment and checks

**Adults older than 65 with newly diagnosed glioblastoma who are not suitable for the standard radiotherapy regimen with temozolomide, under NCCP 00461.**

Take temozolomide daily for 21 days during short-course radiotherapy, including weekends as prescribed. After a four-week break, take temozolomide on Days 1–5 of each 28-day cycle for up to six adjuvant cycles.

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | During treatment | Between visits / recovery | Next-cycle / response review | After a pause or stop |
|---|---|---|---|---|---|
| **Blood count / kidneys / liver** NCCP 00461 | FBC, renal/liver profile, glucose, HBsAg and anti-HBc | FBC weekly during three-week RT | Continue FBC weekly for four weeks after RT; renal/LFT at 1 and 4 weeks after RT | Day 1 FBC/renal/LFT; cycle 1 can use four-week post-RT review. EMA Day 22 FBC | LFT after cycles; follow delayed toxicity after the last dose |
| **Neurology / medicines / function** NCCP 00461 | Record neurological symptoms, daily function and current medicines | Review seizures, headache, cognition, mood, walking and any steroid plan | Report changes early; do not wait for the next MRI | Review treatment tolerance, medication support and patient goals | Agree follow-up and rehabilitation or symptom support |
| **MRI / diagnosis** NCCP 00461 | Confirm integrated diagnosis, molecular results and baseline imaging | MRI timing according to the neuro-oncology plan | Keep the booking and result-review contact in your passport | Interpret scan with symptoms, treatment effects and clinical review | Agree the next imaging and clinical review |

# What each check is for

1 Temozolomide with three weeks of radiotherapy

Daily capsule phase

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

**Ask / act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

2 Four-week break before adjuvant treatment

Recovery and review

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

**Ask / act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

3 Up to six adjuvant 28-day cycles

Five capsule days per cycle

This plan combines temozolomide with a shorter course of radiotherapy, followed by a break and adjuvant temozolomide, to help control newly diagnosed glioblastoma. NCCP identifies this particular indication as unlicensed; your team explains the evidence, alternatives and local consent process.

**Ask / act:** Confirm the current phase, prescription, blood tests and support medicines.

Sources: NCCP 00461

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.38